South African Health Products
Regulatory Authority
Building A
Loftus Park
Arcadia
Pretoria

SIX MONTHLY PROGRESS REPORT FORM

FOR CLINICAL TRIALS

INSTRUCTIONS

1. This form is to be completed six-monthly from the date of approval of the clinical trial by the South African Health Products Regulatory Authority (SAHPRA).

2. The applicant/sponsor must complete Parts A and B.

3. Part A is specifically for the study participants in South Africa.

4. Part B is an overall safety line listing for the study in South Africa (and any other issues of special concern outside South Africa) and includes all Serious Adverse Events (SAEs) and Suspected Unexpected Serious Adverse Reactions (SUSARS) for all participants in this study.

5. *Note:* Protocol Deviations must be reported separately, six-monthly to SAHPRA. Protocol deviations are not part of the report but should be submitted as a separate communication at the same time as this report.

6. The End of Study progress Report should include the summary of investigational product authorized for importation (initial and amendment), quantity imported, used and remaining (destroyed) at end of the study.

   Note: The End of Study report should be submitted 30 days after completion or termination of a Clinical Trial.

CLINICAL TRIAL SIX-MONTHLY PROGRESS REPORT

PART A: STUDY OVERVIEW SOUTH AFRICA	
1 SAHPRA Database tracking number	n/a
2 Study Title	Innovative machine learning and multi-source data analysis towards the development of an urban heat-health Early Warning System for African cities
3 Protocol number	HEAT002
4 *Details of Sponsor / Applicant:*	
4.1 Name of Sponsor	Fogarty International Center and National Institute of Environmental Health Sciences (NIEHS) and OD/Office of Strategic Coordination (OSC) of the National Institutes of Health under Award Number U54 TW 012083
4.2 Name of Applicant	Prof Matthew Chersich
4.3 Contact Person	Mr. Craig Parker
4.3.1 Telephone number	011 3855300
4.3.2 Fax number	
4.3.3 Cell-phone number	+27 792848593
4.3.4 E-mail address	cparker@wrhi.ac.za
5 List of all active trial sites, address and Principal Investigators (PIs)	n/a
6 *Trial Information:*	
6.1 Date of approval of study	06 June 2023
6.2 Treatment hold (if applicable) with reasons (start date and stop date of hold should be included)	n/a
6.3 Expected date of completion	June 2026
7 *Number of participants in the trial (per site), (this section should be accumulative):*	
7.1 Screened (signed consent)	n/a
7.2 Randomised	n/a
7.3 Withdrawn from treatment (continue in follow up), with reasons	n/a
7.4 Withdrawn from study (early termination), with reasons	n/a
7.5 Study completed	n/a
7.6 Lost to follow-up	n/a
7.7 Deaths	n/a

8	Applicant/Sponsor comment on progress to date	We are pleased to report on the considerable strides made in the study screening process for our ongoing project, RP2. **Study Screening:** From the period spanning 2000 to 2022, we embarked on a comprehensive screening of 2267 studies conducted in Johannesburg. The aim was to identify studies that align with our project's eligibility criteria. We were able to identify 42 studies that met our requirements for inclusion. However, we encountered ambiguities with 18 studies, which are currently undergoing a more thorough review to clarify their eligibility. A significant number, precisely 2205 studies, did not meet our eligibility criteria and were consequently excluded. **Eligible Studies by Research Units:** Out of the 42 eligible studies, they are associated with the following prestigious research units: 1. PHRU - Perinatal HIV Research Unit: 14 studies 2. WRHI - Wits Reproductive Health & HIV Institute: 9 studies 3. VIDA - The Vaccine and Infectious Diseases Analytics Research Unit: 6 studies 4. CHRU - Clinical HIV Research Unit: 5 studies 5. Ezintsha: 4 studies 6. DPHRU - Developmental Pathways for Health Research Unit: 1 study 7. ECRU - Effective Care Research Unit: 1 study 8. WCR Bara Clinical Trial Centre: 1 study 9. Lynn Morris - Virology: 1 study **Current Data Transfer Agreements (DTAs):** We are currently collaborating with VIDA and DPHRU to facilitate data transfer agreements for the following studies: 1. **Middle-aged Soweto Cohort (MASC):** This cohort includes men and women who participated in the original AWI-Gen 1 study (2011-2014; n=~2000), and who were followed up again in 2017/2018 as part of a diabetes study (n=~1000). AWI-Gen 2 was then completed in 2021/2022 with a total of 1400 participants. 2. **Surveillance among Healthcare Workers for SARS-Coronavirus–2 Infection (COVID-HCW study):** This study focused on the surveillance of SARS-Coronavirus-2 infection among healthcare workers. 3. **ChadOx1 SARSCoV-2 Vaccine Trial:** This trial examined the effectiveness of the ChadOx1 SARSCoV-2 Vaccine. **Acquired Datasets:**

		Acquired Datasets: 1. **HPTN 082:** This study was conducted from 2016 to 2018, involving 137 young women at Ward 21. The primary focus was to evaluate the acceptance rate, adherence, acceptability, and continuation of oral pre-exposure prophylaxis (PrEP) among young southern African women. 2. **HPTN 075:** Carried out between 2015 and 2017, this prospective cohort study involved 101 men and transgender women at the Soweto HPTN CRS located at the Chris Hani Baragwanath Hospital. The study sought to ascertain the feasibility of recruiting and retaining men who have sex with men (MSM) in a multi-country study in sub-Saharan Africa (SSA). 3. **New Regimens to Prevent Tuberculosis in Adults with HIV Infection:** This randomized controlled trial was designed to evaluate three new regimens for latent tuberculosis (TB) prevention in adults with HIV infection. **Ezintsha Acquisitions:** 4. **D4T:** A randomised, double-blind, multi-centre, parallel-group Phase 3b study that demonstrated the non-inferiority of Stavudine (20 mg twice daily) compared with Tenofovir Disoproxil Fumarate (300 mg once daily) when administered in combination with Lamivudine and Efavirenz in antiretroviral-naive patients infected with HIV-1. 5. **ADVANCE :** A 96-week randomised, Phase 3 non-inferiority study that compared DTG+TAF+FTC with DTG+TDF+FTC and EFV+TDF+FTC in patients infected with HIV-1 starting first-line antiretroviral therapy. 6. **DRV052:** This trial aimed to generate evidence to replace the current standard of care second-line HIV treatment LPV/r with a lower-dose DRV/r when used in combination with 2 nucleoside reverse transcriptase inhibitors (NRTIs).
9	Summary Data Safety Monitoring Board or Safety Committee recommendations	n/a

PART B: OVERALL SAFETY LINE LISTING

10 Safety Line Listing of all Serious Adverse Events (SAEs) and Suspected Unexpected Serious Adverse Reactions (SUSARS) for all participants per site in this study in South Africa

Note: Detailed Site Specific Line listing may be submitted as an attachment (should not be accumulative) as per Safety Reporting guideline

SAEs and SUSARS	Relationship to study medicine (investigator's opinion)	Outcome(s)
Site 1: (name of site)		
n/a	Possible (2) Probable (1) Definite (2) Unrelated (2) unknown (0)	7 patients recovered 2 still treated
Site 2: (name of site)		
Site 3: (name of site)		
Any safety issues of special concern outside South Africa		

11 Line Listing of all critical and major protocol violations at the site:

Protocol *Violation* is any change, divergence, or departure from the study design or procedures defined in the protocol that might significantly affect participants' safety, and well-being and/or the reliability of the study data.

Protocol *Deviation* is accidental or unintentional changes to, or non-compliance with the research protocol that does not increase risk or decrease benefit or does not have a significant effect on the participants, safety or well-being; and/or the reliability of the study data.

Note: South African Site Specific Line Listing of all major protocol violations may be submitted as an attachment

Site specific Protocol Deviations must be reported separately to SAHPRA, six-monthly.

Critical and Major Protocol Violations	**Resolution/Action taken**
Site 1: (name of site)	
n/a	
Site 2: (name of site)	
Site 3: (name of site)	
12 National Principal Investigator comment on other major safety concerns (this should include information impacting on the risk-benefit profile, including changes in nature, severity or frequency of risk factors, *etc.*)	n/a
13 Provide proof of current registration on South African National Clinical Trials Registry (SANCTR)	n/a
13.1 DoH Number	n/a
13.2 Enclose transcript reflecting current information from SANCTR	n/a
14 Provide the summary of investigational product: ▪ Authorized for importation ▪ Imported	n/a

▪ Used during the Clinical Trial ▪ Destroyed (include the destruction certificate(s)) and ▪ Quantity to be exported or exported	
14.1 Overall comment on Investigational Product Reconciliation	n/a
15 Planned date for provision of trial results to SAHPRA and SANCTR (applicable to final progress report)	n/a
We, the undersigned, agree that we have reviewed the above-mentioned report and is accurate. The trial is conducted according to the approved protocol, South African legal, ethical and regulatory requirements. In case of deviation or Violation those are reported accordingly.	
Signature of National Principal Investigator	**Date 04/07/2023** 
Signature of Applicant/Sponsor	**Date 05/07/2023** 

FOR SAHPRA USE ONLY:

Comments:

Action: ☐ **Continue Trial**

☐ **Further information required from Applicant / Sponsor**

☐ **Refer to Clinical Trials Committee and / or Inspectorate**

Reviewed by: ..... **Date:** .....

Signature: .....

Acquired Datasets:

7. **HPTN 082:** This study was conducted from 2016 to 2018, involving 137 young women at Ward 21. The primary focus was to evaluate the acceptance rate, adherence, acceptability, and continuation of oral pre-exposure prophylaxis (PrEP) among young southern African women.
8. **HPTN 075:** Carried out between 2015 and 2017, this prospective cohort study involved 101 men and transgender women at the Soweto HPTN CRS located at the Chris Hani Baragwanath Hospital. The study sought to ascertain the feasibility of recruiting and retaining men who have sex with men (MSM) in a multi-country study in sub-Saharan Africa (SSA).
9. **New Regimens to Prevent Tuberculosis in Adults with HIV Infection:** This randomized controlled trial was designed to evaluate three new regimens for latent tuberculosis (TB) prevention in adults with HIV infection.

Ezintsha Acquisitions:

10. **D4T:** A randomised, double-blind, multi-centre, parallel-group Phase 3b study that demonstrated the non-inferiority of Stavudine (20 mg twice daily) compared with Tenofovir Disoproxil Fumarate (300 mg once daily) when administered in combination with Lamivudine and Efavirenz in antiretroviral-naive patients infected with HIV-1.
11. **ADVANCE :** A 96-week randomised, Phase 3 non-inferiority study that compared DTG+TAF+FTC with DTG+TDF+FTC and EFV+TDF+FTC in patients infected with HIV-1 starting first-line antiretroviral therapy.
12. **DRV052:** This trial aimed to generate evidence to replace the current standard of care second-line HIV treatment LPV/r with a lower-dose DRV/r when used in combination with 2 nucleoside reverse transcriptase inhibitors (NRTIs).

UPDATE HISTORY

Date	Reason for Update	Version & Publication
November 2017	First version published for implementation	Version 1, January 2018
May 2018	Changes to sections 1, 4, 5, 6, Part A, Part B, including change from MCC to SAHPRA	Version 2, June 2018
April 2020	Administrative	Version 3, April 2020
June 2022	Addition of sections 13, 14 and 15 Document number changed from 6.27 to GLF-CEM-CT-06A	Version 4, June 2022